

St. John's Medical College Hospital

Sarjapur Road, Koramangala, Bengaluru - 560034, Karnataka, India

Tel: +91-80-2206-5000 | Reg No: KA-MED-1963-00045

PATIENT DISCHARGE SUMMARY & CLINICAL RECORD

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SECTION 1 — PATIENT IDENTIFICATION

Patient Full Name	Patient ID (MRN)
Mohammed Irfan Shaikh	SJMC-BLR-2024-11209
Date of Birth	Age
03/11/1962	61 years
Sex	Blood Group
Male	A+
Residential Address	
88/2, 5th Main Road, Frazer Town, Bengaluru - 560005, Karnataka, India	
Contact Number	Emergency Contact
+91-99000-87654	Fatima Shaikh (Spouse) — +91-99000-23456
Insurance ID	Nationality
NIACL-KA-MED-2024-334512	Indian

SECTION 2 — ADMISSION DETAILS

Admission Date	Discharge Date
02/11/2024	09/11/2024
Ward / Unit	Attending Physician
Oncology Ward — Bed 12C	Dr. Deepa Krishnamurthy (MD, DM Medical Oncology)
Referring Physician	Department
Dr. Salim Hussain (Pulmonologist, Bengaluru)	Medical Oncology
Reason for Admission	

Evaluation of progressive dyspnea, hemoptysis, and significant weight loss (12 kg in 3 months). CT chest showed 4.2 cm right upper lobe mass.

SECTION 3 — PRESENTING SYMPTOMS

- Progressive dyspnea on exertion (2 months), now at rest
- Hemoptysis — blood-streaked sputum (3 episodes in 2 weeks)
- Significant weight loss — 12 kg over 3 months
- Persistent dry cough (4 months)
- Right-sided chest pain, dull aching
- Fatigue and anorexia
- Hoarseness of voice (2 weeks)

Symptom Onset	Duration
01/08/2024	4 months progressive

SECTION 4 — DIAGNOSIS

Primary Diagnosis

Non-Small Cell Lung Cancer (NSCLC) — Adenocarcinoma, Stage IIIB

Secondary Diagnosis

COPD (moderate), Ex-smoker (40 pack-years)

ICD-10 Code

C34.1 — Malignant neoplasm of upper lobe, bronchus or lung

SECTION 5 — LABORATORY RESULTS

Marker	Value	Unit	Reference Range	Flag
CEA (Tumor Marker)	142	ng/mL	< 5.0	HIGH
CYFRA 21-1	18.4	ng/mL	< 3.3	HIGH
WBC	11,800	cells/uL	4,500–11,000	HIGH
Hemoglobin	9.6	g/dL	13.5–17.5	LOW
Albumin	2.9	g/dL	3.5–5.0	LOW
LDH	412	U/L	140–280	HIGH
Calcium	11.2	mg/dL	8.5–10.5	HIGH
Creatinine	1.1	mg/dL	0.7–1.2	NORMAL
Platelet Count	420,000	cells/uL	150,000–400,000	HIGH

SECTION 6 — MEDICATIONS ADMINISTERED

Medication	Dosage	Frequency	Response
Carboplatin	AUC 5 IV	Every 3 weeks (Cycle 1)	Ongoing
Pemetrexed	500 mg/m2 IV	Every 3 weeks	Ongoing
Dexamethasone	8 mg	Pre-chemo (3 days)	Good response
Ondansetron	8 mg IV	TDS during chemo	Good response
Folic Acid	400 mcg	Daily (supplementation)	Prophylactic
Vitamin B12	1000 mcg IM	Every 9 weeks	Prophylactic
Morphine SR	10 mg	Twice daily	Good pain relief

SECTION 7 — PROCEDURES & IMAGING

- CT-guided biopsy of right upper lobe mass — Adenocarcinoma confirmed
- PET-CT scan — Mediastinal lymph node involvement (N2), no distant metastasis
- Bronchoscopy with BAL — Malignant cells in washings
- EBUS (Endobronchial Ultrasound) — Mediastinal staging
- Pulmonary Function Tests — FEV1 58% predicted
- Cycle 1 Chemotherapy — Carboplatin + Pemetrexed administered
- Bone marrow biopsy — No marrow involvement

Imaging Metadata

CT Chest (28/10/2024): 4.2 x 3.8 cm spiculated mass right upper lobe. Mediastinal LAP. PET-CT (30/10/2024): FDG-avid RUL mass (SUVmax 12.4). N2 disease. No M1. MRI Brain (01/11/2024): No intracranial metastases. Chest X-ray (02/11/2024): RUL opacity with mediastinal widening.

SECTION 8 — GENOMIC INFORMATION (Optional)

Genomic Findings

EGFR mutation — Exon 19 deletion detected (sensitizing mutation). ALK rearrangement — Negative. PD-L1 TPS: 35%. KRAS — Wild type. TMB — Intermediate (8 mut/Mb). Recommendation: Consider Osimertinib targeted therapy post-chemotherapy evaluation.

SECTION 9 — CLINICAL OUTCOME

Discharge Outcome	Follow-up Required
Ongoing treatment — Stable after Cycle 1	Yes — Oncology OPD in 3 weeks for Cycle 2. Repeat CT chest after 2 cycles.

Discharge Instructions

Monitor for neutropenic fever — report temperature > 38C immediately. High protein diet. Anti-emetics as needed. No NSAIDs (renal protection during chemo). Palliative care team referral arranged. Genetic counselling for family.

SECTION 10 — CLINICAL NOTES

Physician Clinical Notes

61-year-old male ex-smoker presented with 4-month history of respiratory symptoms. CT revealed 4.2 cm RUL mass. Biopsy confirmed adenocarcinoma. PET-CT staged as IIIB (T3N2M0). EGFR exon 19 deletion detected — targetable mutation. Multidisciplinary tumour board discussed case — decision for concurrent chemoradiation with carboplatin/pemetrexed, followed by osimertinib maintenance if response confirmed. Cycle 1 administered without major toxicity. Hemoglobin low — iron supplementation started. Patient and family counselled extensively on diagnosis, treatment plan, and prognosis. Palliative care team involved for symptom management. ECOG PS 2 at discharge.